

Knee and Lower Leg Conditions Disability Benefits Questionnaire

SYNTHETIC REPLICA for software testing. Structure follows VA Form 21-0960M-9. All data fictional.

Form number

Veteran name

Date of birth

SSN

Date of examination

Examiner

Knee examined ☒ Right ☐ Left

Section 1. Diagnosis

Diagnosis #1

ICD code

Section 2. Medical history

History (onset, course, treatment)

Veteran reports right knee pain beginning in service after a fall during a field exercise in 2011, treated conservatively. Symptoms have

Section 3. Flare-ups

Does the Veteran report flare-ups? ☒ Yes ☐ No

Describe (frequency, duration, severity, functional loss)

Weekly flares lasting 1-2 days after prolonged walking; severity moderate.

Section 4. Functional loss or impairment

Functional loss or impairment reported? ☒ Yes ☐ No

Describe

Difficulty with prolonged standing, squatting, and stairs.

Section 5. Initial range of motion (ROM) measurements

Normal knee ROM: flexion 0 to 140 degrees; extension 140 to 0 degrees (38 CFR 4.71, Plate II).

Flexion (0-140), degrees

Extension (140-0), degrees short of 0

Is ROM abnormal or outside normal range?

☒ Yes ☐ No

If abnormal, does ROM contribute to functional loss?

☒ Yes ☐ No ☐ Not addressed

Is pain noted on examination (on ROM)?

☒ Yes ☐ No

Does the pain cause functional loss?

☒ Yes ☐ No ☐ Not addressed

Evidence of pain with weight-bearing?

☒ Yes ☐ No ☐ Not tested

Evidence of pain in nonweight-bearing?

☒ Yes ☐ No ☐ Not tested

Evidence of pain on passive ROM testing?

☒ Yes ☐ No ☐ Not tested

If not tested, explain

Objective evidence of localized tenderness or pain on palpation?

☒ Yes ☐ No

Objective evidence of crepitus?

☒ Yes ☐ No

Is the opposing joint undamaged?

☒ Yes ☐ No

Opposing joint flexion, degrees

Opposing joint extension, degrees

Section 6. Repetitive-use testing

Was repetitive-use testing (3 repetitions) performed?

☒ Yes ☐ No

If not performed, explain

Flexion after 3 repetitions, degrees

Extension after 3 repetitions, degrees

Additional loss of function or ROM after repetitions?

☒ Yes ☐ No

Section 7. Flare-ups and repeated use over time

Is the examination being conducted during a flare-up?

☐ Yes ☐ No

Do pain, weakness, fatigability or incoordination significantly limit functional ability during flare-ups?

☐ Yes ☐ No ☐ Unable to say without...

Estimated flexion during flare-ups, degrees

40

Estimated extension during flare-ups, degrees

0

If unable to estimate, explain

Contributing factors of disability (check all that apply):

- ☒ Pain on movement
- ☐ Weakened movement
- ☐ Excess fatigability
- ☐ Incoordination
- ☒ Less movement than normal
- ☐ Swelling
- ☐ Atrophy of disuse
- ☐ Instability of station

Section 9. Muscle strength testing

Knee flexion strength (0/5 - 5/5)

5/5

Knee extension strength (0/5 - 5/5)

5/5

Muscle atrophy?

☐ Yes ☐ No

Section 10. Ankylosis

Ankylosis of the knee?

☐ Yes ☐ No

Section 11. Joint stability tests

Anterior instability (Lachman test)

☐ Normal ☐ 1+ ☐ 2+ ☐ 3+
☐ Not tested

Posterior instability (posterior drawer test)

☐ Normal ☐ 1+ ☐ 2+ ☐ 3+
☐ Not tested

Medial instability (valgus pressure)

☐ Normal ☐ 1+ ☐ 2+ ☐ 3+
☐ Not tested

Lateral instability (varus pressure)

☐ Normal ☐ 1+ ☐ 2+ ☐ 3+
☐ Not tested

History of recurrent subluxation

☐ None ☐ Slight ☐ Moderate ☐ Severe

History of lateral instability

☐ None ☐ Slight ☐ Moderate ☐ Severe
Section 11b. Ligament and patellar instability (DC 5257, 2021 criteria)

Ligament injury (cruciate or collateral)

☐ None ☐ Sprain ☐ Incomplete tear
☐ Complete tear

If a tear: repair status

☐ N/A ☐ Repaired ☐ Unrepaired
☐ Failed repair

Does the injury cause persistent instability?

☐ Yes ☐ No

Has a medical provider prescribed bracing for ambulation?

☐ Yes ☐ No

Assistive device prescribed by a medical provider for ambulation

☐ None ☐ Cane ☐ Crutches ☐ Walker

Diagnosed patellofemoral complex condition with recurrent patellar instability?

☐ Yes ☐ No

If yes: history of surgical repair of the patellofemoral complex?

☐ Yes ☐ No

Section 18. Diagnostic testing

Imaging studies performed and reviewed?

☒ Yes ☐ No

Degenerative or traumatic arthritis documented?

☐ Yes ☒ No

Section 19. Functional impact

Impact on ability to work

The knee condition limits prolonged standing and walking; the Veteran can perform sedentary work with position changes.

Section 20. Remarks

Remarks, if any

Mr. Naeole (DOB 03/14/1975) reports flares worsen with cold weather.

Medical opinion (if requested)

Was a medical opinion requested?

☒ Yes ☐ No

Opinion

☒ At least as likely as... ☐ Less likely than not ...
☐ Cannot be determined ...

Rationale

The current right knee condition is at least as likely as not related to the documented in-service fall because the service treatment reco